RAINBOW CHILDREN'S HOSPITAL & BIRTHRIGHT

Department of Pediatric Radiodiagnosis & Imaging
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FINAL VERIFIED REPORT

ACC: #RCH-PN-94551
UHID: RCH-2026-88142

Patient Name:	**Master Aarav Sharma**	**Exam Date & Time:**	28/10/2026 • 11:20 AM
Age / Sex:	2 Years 4 Months • Male	**Referring Doctor:**	Dr. Ananya Kulkarni, MD (Pediatrics)
Study:	**Chest Digital Radiography (AP View)**	**Clinical History:**	Cough, rapid breathing, low-grade fever × 3 days

TECHNIQUE

Single frontal AP digital radiograph of the chest obtained in pediatric supine/erect position with arms extended overhead. Lead marker 'R' indicates the anatomical right side.



Key Series 1 / Slice 1
Chest AP View (Pediatric)
Marker: Right (R)

FINDINGS

- **Lungs:** Moderate bilateral lung hyperinflation with flattening of both hemidiaphragms. Prominent peribronchial cuffing and increased perihilar bronchovascular markings bilaterally.
- **Parenchyma:** Mild subsegmental atelectatic changes in the perihilar zones. No focal dense lobar consolidation.
- **Pleura & Mediastinum:** Both costophrenic angles and cardiophrenic recesses are clear. No pneumothorax or pleural effusion. Normal pediatric cardiac silhouette.
- **Bones & Soft Tissues:** Rib cage and thoracic cage morphology unremarkable. Normal age-appropriate thymic outline.

IMPRESSION

1. **Bilateral lung hyperinflation with peribronchial thickening:** Features are suggestive of acute viral bronchiolitis / reactive small airway disease.
2. **No evidence of lobar pneumonia, effusion, or pneumothorax.**

ADVICE / CLINICAL CORRELATION

Correlate clinically with SpO_2 and respiratory rate. Nebulization and supportive pediatric care as per clinical protocol.

Dr. Rajesh Deshmukh, MBBS, DMRD, DNB
Consultant Pediatric Radiologist
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